



TIRZEPATIDE RESEARCH HUB · RESEARCH USE ONLY

BURN THE FAT. GUARD THE HEART.

FOR MEN & WOMEN. Burn fat and keep your heart healthy.

Get G2-T 30 mg at BioLinx

Read the guide first

Also shown to help with: diabetes · insulin resistance · appetite control & binge eating · pre-diabetes · pcos

Once per week · at night · Difficulty: beginner

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RESEARCH SHOWED THAT...

20% body weight
lost in under 18 months.

NEJM, 2025

People on tirzepatide lost
nearly twice the weight of
people on Ozempic.

NEJM, 2021



STEP 1 OF 4 · WHAT TO BUY

SHOPPING LIST

Easy-to-follow shopping list to safely practice Tirzepatide. Match your commitment level — don't skip anything.



PEPTIDE · ONE ORDER

G2-T 30 MG

Burn fat and keep your heart healthy.
Common starting dose: **1.25 mg**, once per week, at night.

Get G2-T 30 mg at BioLinx

biolinxlabs.com/products/g2-t-30-mg

HOW MANY VIALS TO BUY · BEST VALUE BY GOAL

GOAL	VIAL	QTY	WHERE
1 MONTH	10 mg vial	1×	<u>See it</u> biolinxlabs.com/products/g2-t-10-mg
4 MONTHS RECOMMENDED	30 mg vial	4×	<u>See it</u> biolinxlabs.com/products/g2-t-30-mg
FULL CYCLE	30 mg vial	7×	<u>See it</u> biolinxlabs.com/products/g2-t-30-mg

MATERIALS ≈ \$41 ON TOP — FULL LIST ON THE NEXT PAGE



PRODUCTS ARRIVED?

QUICK SUMMARY OF YOUR LAB ITEMS:

TWO VIALS IN THE BOX • G2-T 30 MG + BACTERIOSTATIC WATER 10 ML

VIAL 1 **G2-T 30 mg.** Arrives as a powder. Don't touch it until you've read page 4.

VIAL 2 **Bacteriostatic water — 10 mL.** This is what turns the powder into a liquid. Sterility and pH matter — don't substitute.

GROCERY LIST • ≈ \$41

ITEM	SPEC	QTY	WHERE
INSULIN SYRINGES BUY	1cc U100, 30G, 5/16"	50–100 ct	Any pharmacy
ALCOHOL PREP PADS BUY	Any brand, 70% isopropyl	50–100 ct	Any pharmacy
SHARPS CONTAINER BUY	1 quart, for needle disposal	1×	Any pharmacy
BAC WATER BUY	Bacteriostatic water, 10 mL — sterility and pH matter, don't substitute	1 vial	See it biolinxlabs.com/products/bacteriostatic-water-10-ml

Any brand works for syringes, pads and sharps — but not BAC water. Use 2 fresh syringes and 2 fresh pads every session: one to mix, one to inject.

HOW TO STORE IT

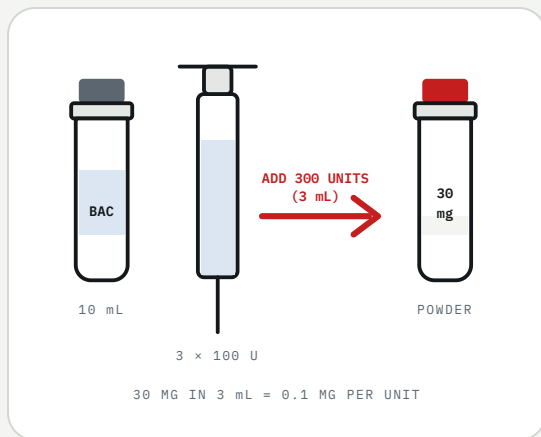
POWDER	Freezer — lasts years
RECONSTITUTED	Fridge — about 28 days. Write the mix date on the vial.
WATER	Room temperature — check the expiration date



STEP 2 OF 4 · RECONSTITUTION – FROM POWDER TO LIQUID

LET'S “FORM” YOUR PEPTIDE!

“Form” means “reconstitute”. Your product arrives as a powder — you need to liquify it before “you begin your research.” 100 units = one full syringe. **10 mg vial → add 100 units (1 mL) · 30 mg vial → add 300 units (3 mL).** Every dose in this guide assumes exactly that ratio, so measure it — don't eyeball it.



YOUR SIMPLIFIED UNITS

DOSE FILL TO

1.25 mg 12.5 units**2.5 mg** 25 units**5 mg** 50 units**7.5 mg** 75 units**10 mg** 100 units**12.5 mg** 125 units – 100 + 25**15 mg** 150 units – 100 + 50

Change the vial, water or dose → [interactive calculator](#)

1**Remove the plastic caps & wipe both rubber vial tops**

Fresh alcohol pad on the G2-T vial and the bac water vial.

2**Draw 300 units of bac water**

That's 3 full pulls on a 100-unit insulin syringe (3 mL total).

3**Inject it slowly down the side of the 30 mg vial**

Let it run down the glass. Don't blast the powder.

4**Swirl gently. Never shake. Place it in the fridge!**

Slow and gentle wins this one — shaking wastes peptide. Discard that syringe into the sharps container. Let it rest until completely clear (36–46 °F); use within ~28 days; never freeze it.



STEP 3 OF 4 · INJECT

WHERE TO INJECT

Time to inject · clean, pinch, poke, done. Once per week, at night.

1

Take G2-T out of the refrigerator and extract your dose

Your first dose is **1.25 mg = 12.5 units** on a 100-unit syringe. Wipe the vial top with a fresh alcohol pad, let it air-dry a few seconds, then draw your units.

2

Pinch a fold of skin on the stomach + use an alcohol wipe

Two inches away from the belly button. Wipe with a fresh alcohol pad, let it air-dry, and don't touch that spot again before injecting. Rotate sites.

3

Needle in at the base of the fold

Push the plunger slowly and steadily. Hold for a few seconds, then withdraw.

4

That's it — you just did your first shot!

Drop the needle straight into the sharps bin. Vial goes back in the fridge.



STOMACH · PINCH A FOLD · NEEDLE AT THE BASE

WATCH FIRST

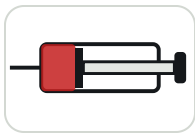
Mixing and injecting from start to finish, on video: professorpeptides.com. Use 2 fresh syringes and 2 fresh pads every session — one to mix, one to inject.



STEP 4 OF 4 · DOSING & TIMING SCHEDULE

TIRZEPATIDE SCHEDULE (PROTOCOL)

“Schedule” means “protocol”. Everything below assumes the 30 mg vial was reconstituted with exactly 300 units (3 mL), read off a standard 100-unit insulin syringe.



FIRST DOSE

How much, how often & when?

WHEN

At night. Once per week.

HOW MUCH

1.25 mg to start.

Dose steps up week by week with no breaks — this is called **titration**. Full schedule on page 9.

UNITS

Extract **12.5 units** on a 100-unit insulin syringe.

12.5 units = 1.25 mg. Exact variables: 100-unit syringe · 30 mg vial reconstituted with 300 units · 0.1 mg per unit.

HOW OFTEN

Once per week. Hold each step for its full block before moving up.

RUN LENGTH

16-week minimum cycle.

Don't rush it — start at the lowest amount, hold each step for its full block, and only step up if the current dose is comfortable. Most people never need the top doses.



STEP 4 OF 4 · DOCUMENT

WHAT TO EXPECT + SIDE EFFECTS

Mostly mild, mostly early, mostly right after a dose increase. Know the three tiers before the first shot, not after.

NORMAL / EXPECTED

First few days

- Reduced appetite and feeling full fast — this is tirzepatide working
- Mild nausea, burping, heartburn or reflux, especially the first 1–2 weeks after a dose increase
- Constipation or loose stools
- Redness or a small bump at the injection site; mild fatigue

COMMON, WORTH NOTING

Manage it, don't push the dose

- Nausea was the most reported effect in the trials — it rose sharply when people started too high or stepped up too fast
- Occasional vomiting or diarrhea; headache or dizziness
- A slightly faster resting heart rate that persists during treatment
- Hair shedding or tiredness with rapid weight loss — eat enough protein

STOP & CALL A DOCTOR

Don't push through it

- Severe, persistent stomach pain — especially spreading to the back, with or without vomiting (possible pancreatitis)
- Pain under the right ribs, fever, or yellowing skin/eyes (gallbladder)
- Vomiting that stops you keeping fluids down — dehydration risk
- A lump or swelling in the neck, hoarseness, trouble swallowing
- Signs of an allergic reaction — swelling, rash, difficulty breathing
- Shakiness, sweating or confusion if you also take insulin or blood-sugar medication (low blood sugar)

NOTE: for this one it is **HIGHLY RECOMMENDED** to start at the lowest amount first and work up over time, as needed. Not for anyone pregnant, breastfeeding or planning pregnancy, anyone with a personal or family history of medullary thyroid cancer or MEN 2, or a history of pancreatitis — talk to a doctor before use.



FIRST-TIME TL;DR

RECAP: CHECKLIST

Everything from pages 2–7, in the order it happens.

BEFORE

- ☐ Vial reconstituted with exactly 300 units of bac water, resting in the fridge, mix date written on it
 - ☐ Supplies laid out: new syringe, alcohol pad, sharps container
 - ☐ Dose set at **1.25 mg = 12.5 units** — not higher, no matter what a forum says
-

DURING

- ☐ Inject once per week, at night — stomach, rotate the spot
 - ☐ Note the date and dose somewhere — a phone note is fine
-

AFTER

- ☐ Expect possible redness, mild irritation or a headache — it settles
 - ☐ Mild nausea can happen after a dose increase — sit tight, it typically passes
 - ☐ If anything feels more than mild — dizziness, real nausea, allergic reaction — stop and get medical advice
-

NEXT BLOCK

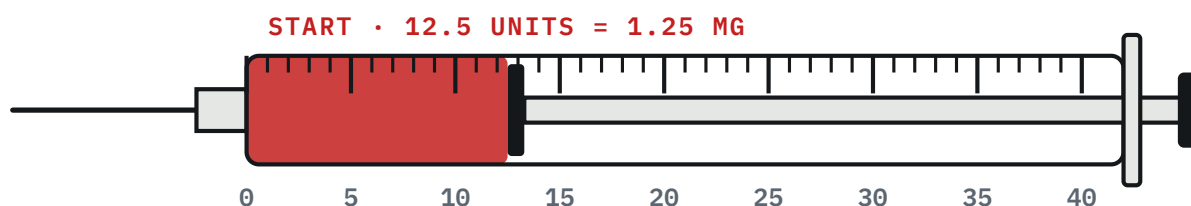
- ☐ Hold each step for its full block before stepping up — never skip a step (16-week minimum cycle)
 - ☐ Only step up if the current dose is tolerated; if side effects are rough, stay another block
 - ☐ Fresh syringe, fresh pad, needle in the sharps bin — every single time
-

UNIT CHEAT-SHEET · 100-UNIT SYRINGE · 30 MG VIAL RECONSTITUTED WITH 300 UNITS

HOW TO TITRATE (TITRATION SCHEDULE)

Assumes the 30 mg vial was mixed with exactly 300 units, so **1 unit = 0.1 mg**.

Units shown are per shot.



WHEN	DOSE	SYRINGE · PER SHOT	NOTES
Month 1	1.25 mg per week	12.5 units	START HERE
Month 2	2.5 mg per week	25 units	
Month 3	5 mg per week	50 units	
Month 4	7.5 mg per week	75 units	IDEAL DURATION
Month 5	10 mg per week	100 units	
Month 6	12.5 mg per week	125 units – 100 + 25	
Month 7	15 mg per week	150 units – 100 + 50	MAX DOSE

Rule of thumb: don't move up a step until you know how the current dose feels. Only step up if the previous block was tolerated. Most people never need to go past the middle of the ramp.



THE G2-T SOURCING GUIDE

PROFESSOR PEPS: SOURCING GUIDE (APPENDIX)

How to verify any supplier in 90 seconds. A legitimate source clears all three checks below in under two minutes — no back-and-forth required.



1

Ask for the COA

A batch-specific Certificate of Analysis — not a generic PDF reused across batches — showing purity $\geq 98\%$ via HPLC/MS.

2

Match the lot number on the vial

It must match the lot number on the COA. If they don't match, walk away.

3

Check the track record

Searchable reviews or forum mentions older than roughly six months. Brand-new storefronts with no history are the highest-risk category.

DON'T WORRY — WE'VE ALREADY DONE THE VETTING

BioLinx Labs clears the list: third-party HPLC testing, a 99%+ purity guarantee and a COA included with every order. biolinxlabs.com/best-sellers

biolinxlabs.com/best-sellers

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Tirzepatide Research Hub · companion to the email series